

Urethral Bulking

Patient Information · An injection treatment for stress urine leakage

WARWIKI

Urethral bulking treats **stress urinary incontinence**, which is leaking when you cough, laugh or move. A soft material is injected into the wall of the urethra to help it close. There is no cut. It is often done in the office with numbing medicine.

How It Works

The material is placed into the wall of the urethra at a few points. This **plumps up** the wall so the channel closes more fully. When pressure in your belly rises, less urine escapes.

Benefits and Risks in Numbers

about
3 in 4

women are cured or better 1 year later; about half have no stress leaks

4–10 in 100

get a bladder infection afterward

about
3 in 100

cannot urinate right afterward and need a catheter for a short time

Numbers are averages from published studies. Your own risk may differ; ask your team.

Is It Right for Me?

- A good choice if you want a **lower-risk** treatment with a quick recovery
- Helpful if you cannot have, or prefer to avoid, a sling
- It works **less well** than a sling, and **repeat injections are common**. A sling is still possible later.
- For men, bulking rarely cures leakage.

Learn the Terms

Stress incontinence

Leaking urine with pressure, such as coughing, laughing, lifting or exercise.

Urethra

The tube that carries urine out of the body.

Bulking agent

The soft material injected to plump the wall of the urethra. It stays in your body.

Cystoscope

The thin camera used to guide the injection.

Top-up

A second injection to add more material.

Sling

A support placed under the urethra during surgery. It lasts longer.

Will it hurt? It is often done in the office with **numbing medicine**, though some people have sedation or anesthesia. Most people feel only mild pressure. Brief burning afterward is common.

Same day

You urinate before you leave to show your bladder empties.

First few days

Mild burning, urgency or a little blood in the urine.

4 weeks or later

A top-up injection, if needed.

Later

If leakage returns, the injection can be repeated.

How to Get Ready

- A **urine test** first. Any infection is treated before the injection.
- Tell your team about **blood thinners**, allergies, and any earlier bulking injection.
- Also tell them if you have diabetes, take medicine that weakens the immune system, or have a genital herpes outbreak.
- If you will have sedation, arrange a ride home.

What Happens

- 1 Numbing gel or a small numbing shot is used.
- 2 A thin **cystoscope** is passed to see the urethra.
- 3 The material is injected at a few points in the wall of the urethra.
- 4 You urinate before you leave. If needed, the bladder is emptied with a small, soft catheter.

Call the office that did your procedure if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- A weak stream, or trouble emptying your bladder
- Pain that gets worse, or heavy bleeding

Go to an emergency department if:

- You cannot urinate for several hours and your bladder feels full or painful. Tell them you had bulking and ask for a small, soft catheter to drain the bladder. A catheter left in can push the material out of place.
- You cannot reach the office and are worried

Three Things to Remember

1. Bulking is a quick injection with no cut that helps the urethra close.
2. It is lower risk than surgery but works less well, and repeat injections are common.
3. Keep a record of the product used. On later scans it can look like a stone or a growth.